

## CDSS Sample Literature Upload

### Citation:

Lee T, Navarro P, Iqbal S, et al.  
Anticoagulation in Atrial Fibrillation with Chronic Kidney Disease  
Stage 3: Comparative Effectiveness and Safety.  
Journal of Translational Cardio-Renal Medicine. 2026;14(2):101–118.  
DOI: 10.5555/jtcrm.2026.142101

### Study Type:

Multicenter retrospective cohort with propensity-score matching.

### Population:

- Adults with non-valvular atrial fibrillation and CKD stage 3.
- N = 8,420 patients from 12 health systems.
- Mean follow-up: 22 months.

### Interventions:

- Group A: warfarin-based anticoagulation.
- Group B: direct oral anticoagulant (DOAC) strategy (apixaban-dominant).

### Primary Outcome:

- Composite of ischemic stroke or systemic embolism.

### Key Secondary Outcomes:

- Major bleeding.
- Intracranial hemorrhage.
- All-cause hospitalization.
- All-cause mortality.

### Structured Abstract:

#### Background:

Patients with atrial fibrillation and CKD stage 3 have competing risks of thromboembolism and bleeding, making anticoagulant selection clinically complex.

#### Methods:

Investigators used propensity-score matching on age, sex, CHA2DS2-VASc score, baseline eGFR, prior bleeding history, and antiplatelet co-therapy. Time-to-event analyses were performed with Cox proportional hazards models.

#### Results:

- Stroke/systemic embolism: hazard ratio 0.92 (95% CI 0.81–1.05), DOAC vs warfarin.
- Major bleeding: hazard ratio 0.74 (95% CI 0.65–0.84), favoring DOAC.
- Intracranial hemorrhage: hazard ratio 0.63 (95% CI 0.47–0.85), favoring DOAC.
- Benefit was most consistent in patients with baseline eGFR 35–55 mL/

min/1.73m<sup>2</sup>.

#### Conclusions:

In AF with CKD stage 3, DOAC-based therapy showed similar thromboembolic protection and lower major bleeding than warfarin in real-world practice. Findings support individualized anticoagulation with strong attention to renal monitoring and interacting medications.

#### Clinical Interpretation:

1. DOAC therapy may be preferred in many CKD stage 3 patients with AF when not contraindicated.
2. Warfarin remains appropriate for selected patients requiring tight titration or with cost/access constraints.
3. Concomitant antiplatelet exposure increased bleeding risk in both groups.
4. Renal trajectory and interaction burden should be re-evaluated at each follow-up visit.

#### Limitations:

- Observational design with residual confounding risk.
- Predominantly urban tertiary-center data.
- Limited subgroup power for eGFR below 35.

#### Keywords:

atrial fibrillation; chronic kidney disease stage 3; anticoagulation; DOAC; warfarin; major bleeding; stroke prevention

#### Data Use Note:

This is synthetic sample literature content intended for ingestion and retrieval validation workflows.